

Question:

A 37-year-old primigravid woman at 36 weeks' gestation is admitted to the hospital 30 minutes after the onset of labor. On arrival, contractions occur every 8–10 minutes. During the last 2 days she has noted decreased fetal movements. The pregnancy had been complicated by gestational hypertension. Current medications include labetalol and a pregnancy multivitamin. Her temperature is 36.8°C (98.2°F), pulse is 94/min, and blood pressure is 154/96 mm Hg. On pelvic examination, the cervix is 40% effaced and 2 cm dilated; the vertex is at -2 station. The uterus is consistent in size with a 30-week gestation. Ultrasonography shows the fetus in vertex position and a decreased amount of amniotic fluid. A fetal heart tracing is shown. Which of the following is the most likely diagnosis? Answer Choices: A. Umbilical cord prolapse B. Umbilical cord compression C. Physiologic fetal heart rate pattern D. Placental insufficiency

Reasoning + Answer:

<think> To approach this reasoning problem, we need to analyze the context of the question, which is centered around a diagnosis in a clinical scenario involving a pregnant woman experiencing decreased fetal movements and signs of gestational hypertension. The most relevant answer here is "placental insufficiency," as it directly addresses the decreased amniotic fluid and fetal heart tracing issues, which are indicative of inadequate placental function. This condition is a serious complication of pregnancy that can lead to fetal distress and is consistent with the clinical presentation described.</think><answer>Placental insufficiency</answer>

